

VISIT · CCTSI, UNIVERSITY OF COLORADO ANSCHUTZ MEDICAL CAMPUS · 7 OCTOBER 2026

Valuing the Hub Function

Crediting support for other institutions' research



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Where we stand

It shapes what we hope to learn.

Hospital

The University of Tokyo Hospital is one of Japan's 16 Core Clinical Research Hospitals.
The criteria I describe today apply to the center I run.

National

Representative Secretary, NUH-CRPI
The council of all national university hospitals: 42 universities, 44 hospitals.

Data

Vice Chair, Japan CDISC Coordinating Committee · AMED principal investigator
Clinical data standards. This layer comes next.

Not one hospital's site visit: we came for a national system that is deciding how to measure its research hubs.

Japan's networks, and why a CTSA hub

Japan has national hospital networks, but no hub program.

42 + 44

National university hospitals

A council: 42 universities, 44 hospitals.

It shares practice and speaks with one voice.

✗ Not a funded hub for its members' studies

NHO 140

National Hospital Organization

140 hospitals, with its own ARO and data center.

A network with a center.

... But inside one organization

CCTSI

One hub, a whole state

Funded to serve several universities, hospital systems and community partners.

✓ What Japan does not have

That is why we chose the CCTSI, and why we wanted to see the CTSC before talking.

Japan's Core Clinical Research Hospitals

Created by law in 2015. The Minister approves hospitals that meet published criteria; 16 hold the designation today.

CTSA award (US)

Nature	A grant
Entry	Compete and win
Review	Periodic; the award can end
Steering	A national program office

Core hospital (Japan)

Nature	A legal status
Entry	Meet the published criteria
Review	Kept until it fails the criteria
Steering	No program office, no peer competition

The criteria are the whole instrument, so what they count matters.

What the criteria count

Three things, measured over the previous three years.

8

Trials

Investigator-initiated trials for drug approval, or 4 of them plus 40 other trials

In principle, led by the hospital's own investigator

45

Papers

Peer-reviewed English papers

In principle, the first author is from the hospital

15

Supports

Studies supported at other institutions, per year. Mandatory, but a floor

15 and 50 score the same

A study led from outside counts only if the hospital's ARO takes on **the whole package**: protocol, data management and monitoring.

The hub function is what the instrument measures least.

What happens as a result

What we see from inside the Core Hospitals: our observation, not yet a quantified study.

- ✕ **Small, easy-to-start trials.** Eight in three years is a hard number, and a small trial counts the same as a large one.
- ✕ **More papers.** Forty-five is a hard number too.
- ✕ **Support is penalized.** Every statistician, data manager and dollar moved to another institution's study is not spent on the hospital's own count.

The Ministry knows this. Its review this year keeps support as a floor, because the effort behind support varies too much to be scored.

That is exactly the problem your funders have had to solve.

Three shifts we propose

Re-weight counting, do not abolish it. Each shift has a working counterpart in how NIH funds research infrastructure.

1 Input → outcome

Count trials that finish and report. Judge approvals and guideline changes case by case, over a longer period.

2 In-house → support

Credit research the hub enabled at other institutions as the hub's own achievement.

3 Single site → network

Cross-cutting infrastructure at the core; disease-specific networks carry the rest.

NIH review, in the funding announcements we read (CTSA hubs, TIN, coordinating centers of Duke, the CCTSI and Vanderbilt): no in-house trial counts, no first-author papers, no points or weights. Reviewers judge coordination, data and statistics, consortium contribution and dissemination.

Two layers of NIH money, FY2025

Base layer: CTSA hubs

\$7–11M (¥1.1–1.8B) per hub, small differences

\$0.57B (¥91B)

Competitive layer

coordinating, data and statistical centers

\$1.0B (¥160B) · where institutions differ · what Japan lacks

What we came to ask

Thirteen written questions in seven themes; you have grouped the answers for 10:10. DCRI sharpened three.

Value

Which numbers NCATS actually reacts to, and how you credit research led at partner institutions

Not whether in-house counts are reported: we now know they are not.

Themes A–C

People

The size of the support workforce, and how you keep trained staff

DCRI: culture, and turnover under 6%. What about here?

Theme D

System

How a hub relates to the other research organizations around it, and data standards

Our colleagues wrote the people and data-standards questions, and will follow up.

Themes E–G

So let me go straight to what we most want to learn.

What we most want to learn

One question is about the review; two are specific to you.

1

The review itself

Your UM1 was renewed in 2023 under the current structure. What actually decided it? Since then, which numbers in your annual reports has NCATS ever acted on?

2

Our question three

If hub designation required a minimum number of in-house trials or first-authored papers, how would the CCTSI and its statewide partners change?

3

The hub and the ARO

At Duke, the CTSA hub and DCRI barely meet. How do the CCTSI and CPC Clinical Research relate: shared faculty, shared services, or separate worlds?



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Thank you



Now I would much rather hear from you.